

# CPR Debriefing Report

Simulation Lab · 2026-08-09

Session J8DED1 · PEA Transitioning to VF

Team Leader: None

Guideline: AHA\_2020

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## 1. Scenario summary

|               |                         |
|---------------|-------------------------|
| Session ID    | J8DED1                  |
| Date          | 2026-08-09              |
| Scenario      | PEA Transitioning to VF |
| Scenario type | PEA_to_VF               |
| Team leader   |                         |
| Team size     | 6                       |
| Duration      | 125m 50s                |
| Guideline     | AHA_2020                |

| Overall score | Grade | Findings | Critical | High | Moderate |
|---------------|-------|----------|----------|------|----------|
| 100 / 100     | A     | 0        | 0        | 0    | 0        |

## 2. Domain scores

Scores are derived from AHA 2020 guideline rules applied to SimMan 3G sensor data and audio transcripts. Confidence indicators reflect data completeness — scores marked medium or low confidence should not be over-interpreted.

| Domain                        | Score               | Confidence              | Key finding |
|-------------------------------|---------------------|-------------------------|-------------|
| Cpr Quality                   | 30/100<br>CI: 10–30 | Low_Data confidence     | —           |
| Shock Delivery                | 15/100<br>CI: 8–15  | Partial_Data confidence | —           |
| Drug Administration           | 15/100<br>CI: 6–15  | Low_Data confidence     | —           |
| Rhythm Recognition            | 10/100<br>CI: 1–10  | Low_Data confidence     | —           |
| Reversible Causes (H'S & T'S) | 10/100<br>CI: 0–10  | Low_Data confidence     | —           |
| Team Leadership               | 10/100<br>CI: 0–10  | Low_Data confidence     | —           |

|                    |                    |                     |   |
|--------------------|--------------------|---------------------|---|
| Team Communication | 10/100<br>CI: 0–10 | Low_Data confidence | — |
|--------------------|--------------------|---------------------|---|

### 3. Protocol deviations

|      |              |        |
|------|--------------|--------|
| HIGH | R_ARREST_024 | @ 0:00 |
|------|--------------|--------|

Basic airway management not documented

Guideline reference: AHA 2025 Primary ABCD Survey — Airway  
Finding ID: fnd\_0001

|      |              |        |
|------|--------------|--------|
| HIGH | R_ARREST_025 | @ 0:00 |
|------|--------------|--------|

Bag mask ventilation not documented

Guideline reference: AHA 2025 Primary ABCD Survey — Breathing  
Finding ID: fnd\_0002

|      |              |        |
|------|--------------|--------|
| HIGH | R_ARREST_026 | @ 0:00 |
|------|--------------|--------|

Defibrillator attachment not documented

Guideline reference: AHA 2025 Primary ABCD Survey — Defibrillation  
Finding ID: fnd\_0003

|      |              |        |
|------|--------------|--------|
| HIGH | R_ARREST_028 | @ 0:00 |
|------|--------------|--------|

IV or IO access not documented during resuscitation

Guideline reference: AHA 2025 Secondary ABCD Survey — Circulation  
Finding ID: fnd\_0005

|          |              |        |
|----------|--------------|--------|
| MODERATE | R_ARREST_027 | @ 0:00 |
|----------|--------------|--------|

Advanced airway not placed during resuscitation

Guideline reference: AHA 2025 Secondary ABCD Survey — Airway  
Finding ID: fnd\_0004

## 4. Communication analysis

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*Detailed communication analysis requires audio transcription. Ensure lapel and ceiling mic audio files are provided for full closed-loop communication metrics.*

## 7. Strengths

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*No strengths data available from narrative synthesis.*

## 5. Reflective prompts

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The following questions are mapped to specific findings detected during this session. Use them to guide the debriefing conversation with the team leader.

1. How could the team improve timing and execution for this step?
2. How could the team improve timing and execution for this step?
3. How could the team improve timing and execution for this step?
4. How could the team improve timing and execution for this step?
5. How could the team improve timing and execution for this step?

## 6. Recommendations

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Ranked by clinical impact. Address critical and high-severity findings before moderate ones in the next training session.

1. Document airway opening technique used during resuscitation.
2. Document ventilation method used. Bag mask ventilation should begin immediately after airway opened.
3. Always document defibrillator attachment time for audit purposes.
4. IV or IO access is required for drug administration. Document route used.
5. Consider advanced airway placement for prolonged resuscitation to enable continuous compressions.